



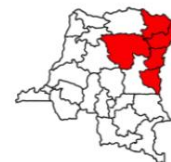
TASK FORCE PRESIDENTIAL EBOLA 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°104/MVEBDB/26/08/2026

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HIGHLIGHTS (last 24 hours)

At the end of the day on August 26, 2026, 81 new confirmed cases of Ebola Virus Disease (EVD), including 35 community deaths, were recorded in the provinces of Ituri (52 cases), North Kivu (22 cases), Haut-Uélé (5), and Tshopo (2). In addition, 7 deaths within Ebola Treatment Centers (ETCs) were recorded.

Two new health zones have been affected in the last 24 hours in North Kivu province.

ACCUMULATIVE CAS 5 794	ACCUMULATIVE DEATH CONFIRMED 2 786	LETHALITY 48,1%	PATIENT IN ISOLATION/CTE 843	ACCUMULATIVE HEALED 1 293	TRACKING RATES CONTACTS (Of the Day) 82,3 %
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo & Lower Uele



60 out of 151
health zones affected
 (39,7%)



Summary of key points from the last 24 hours

In the past 24 hours, **81 new confirmed cases**, including **35 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (52 cases), North Kivu (22), Haut Uélé (5) and Tshopo (2)**.

Two new health zones have been affected in the province of North Kivu, namely those of Biena and Manguredjipa.

As of August 26, 2026, **24 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (12 in Ituri, 11 in Haut Uélé and 1 in North Kivu).

On the other hand, **7 deaths within the treatment center** were recorded, including 5 in Ituri, 1 in North Kivu and 1 in Haut-Uélé. This brings **the total number of deaths for the day to 42**.

From the start of the epidemic until August 26, 2026, **5,794 confirmed cases, including 2,786 deaths**, were reported in the DRC, representing a case fatality rate of **48.1 %**. Ituri remains the epicenter of the epidemic, accounting for 82.9% of cumulative confirmed cases and **64.2%** of new reported cases.

Thus, 60 of the 151 health zones in the six provinces are now affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Distribution of confirmed cases and deaths by affected province

(As of August 26, 2026)

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	52	4802	2 159	45,0%	28/36 (77,8 %)
North Kivu	22	775	527	68,0%	15/34 (44,1 %)
Haut-Uélé	5	194	90	46,4%	6/13 (46,1 %)
Boss	2	17	8	47,1%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas Uélé	0	3	1	33,3%	3/11(27,3%)
Total	81	5 794	2 786	48,1% 60/151 (39,7 %)	

Confirmed cases and deaths by province and health zone (situation as of August 26, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4802	2159	45,0%	52	21	5	26
Give	11	1	9,1%				0
Undersand	7	6	85,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	149	29	19,5%	1	0	0	0
have	2	2	100,0%				0
Bunia	1336	421	31,5%	19	10	0	10
Ladies	27	9	33,3%				0
Drodro	13	6	46,2%				0
Fireworks	73	30	41,1%				0
Gethy	5	2	40,0%				0
Flounder	2	0	0,0%				0
Kilo	32	12	37,5%				0
Team	65	42	64,6%	4	3	0	3
Liter	204	111	54,4%				0
Logo	11	5	45,5%				0
Fight	19	4	21,1%				0
distributed	4	1	25,0%				0
Washing	17	7	41,2%				0
Darkness	33	19	57,6%	1	0	0	0
Mangala	212	114	53,8%	9	5	0	5
Mongbwalu	606	284	46,9%	1	0	0	0
Nanny	199	108	54,3%				0
Nizi	636	254	39,9%	8	2	0	2
Legumes	124	35	28,2%				0

Jungle	10	4	40,0%				0
Rwampara	930	336	36,1%	9	1	0	1
Tchomia	55	25	45,5%				0
A valves	THAT	285	THAT	THAT	THAT	5	5
North Kivu	775	527	68,0%	22	14	1	15
Me	122	87	71,3%	4	2		2
Farm	1	1	100,0%	1	1		1
Butembo	152	116	76,3%	6	6		6
Ten	1	0	0,0%				0
Kalunguta	16	7	43,8%				0
Cut	368	246	66,8%	8	5		5
Kyondo	16	12	75,0%				0
Lubero	1	1	100,0%				0
Be careful.	3	2	66,7%				0
Manguredjipa	1	0	0,0%	1	0		0
Masereka	7	4	57,1%				0
The museums	74	43	58,1%	2	0		0
Mutwanga	1	1	100,0%				0
Oicha	7	5	71,4%	0	0	1	1
Vuh	5	2	40,0%				0
Haut-Uélé	194	90	46,4%	5	0	1	1
Kili Mangbetu	26	14	53,8%				0
Gombari	3	1	33,3%				0
Mathematics	78	32	41,0%	4	0	0	0
Lining	20	16	80,0%	0	0	1	1
Beans	1	0	0,0%				0
Ordinary	66	27	40,9%	1	0	0	0
Boss	17	8	47,1%	2	0	0	0
Bafwasende	1	0	0,0%				0
Kabondo	3	1	33,3%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	8	4	50,0%	2	0	0	0
Mangobo	2	2	100,0%				0
Boss	1	0	0,0%				
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili-Murhesa	3	1	33,3%				0
Bas Uélé	3	1	33,3%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	1	0	0,0%				
Walk	1	0	0,0%				
Total	5794	2786	48,1%	81	35	7	42

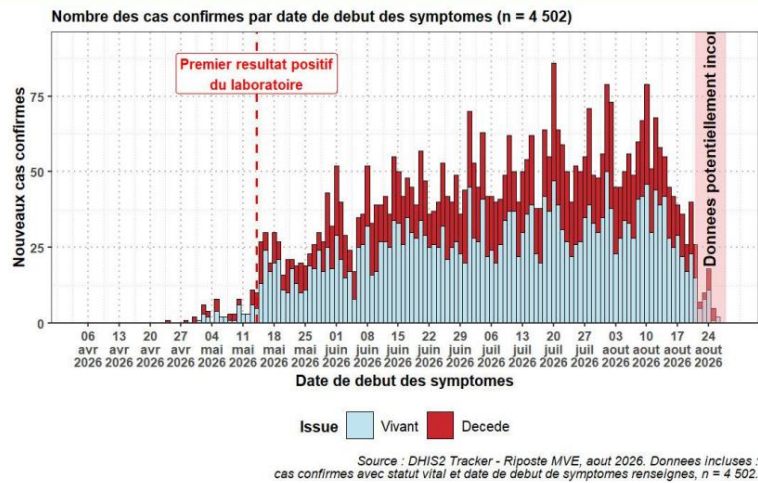
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA : Not Applicable.

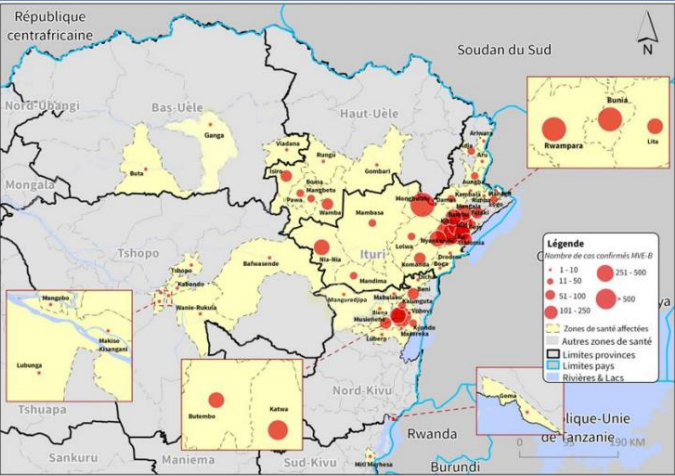
Status of alerts notified by province, as of August 26, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	11	3	14	11	3	0	0	14	10
Bas Uélé	14	1	15	4	1	9	0	5	1
Ituri	660	66	726	197	59	297	59	256	118
North Kivu	1 014	55	1069	98	48	683	7	146	63
South Kivu	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	202	0	202	10	0	192	0	10	8
Total General	1901	125	2 026	320	111	1181	66	431	200

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination
1.1.1. Main Actions

- **South Kivu** : Supervise continued surveillance at the entry points of Uvira (Kavimvira, Kalundu, Kivovo, Beach, Maendeleo and Kasenga); the province remains without new confirmed cases and the last one dates back to June 10, 2026.
- **Bas-Uélé** : Start of vaccination against Ebola virus disease (EVD) with the ERVEBO vaccine; 20 people have been vaccinated in the Buta health zone.

1.1.2. Epidemiological surveillance
1.1.3. Main Actions

By the end of the day on August 26, 2026, 2 026 alerts had been recorded, including 1,678 (82.8%) were verified in the five provinces that submitted their data. At the end of

of these checks, 431 (25.7%) were validated as suspected cases and all were investigated, i.e. 100.0%, of which 200 were transferred to CT/CTE.

The proportion of contacts followed up in the last 24 hours is 82.3% (22,091 seen out of 26,850 to be followed up), below the threshold of 85%: Ituri 85.6% (13,706/16,010), Tshopo 100.0% (308/308), North Kivu 80.5% (6,877/8,541), Haut Uélé 61.0% (1,033/1,694) and Bas Uélé 56.2% (167/297).

1.1.4. Challenges

Poor contact tracing around confirmed cases, mainly in the Lower provinces and Haut Uélé linked to an insufficient number of teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Fourteen (14) alerts were notified to the PoE/PoC in the last 24 hours: three in Ituri at the PoC Mudzi Bala (ZS Bunia), all validated, isolated and referred to the CT Bunia and the CT Elikia; six in North Kivu at the PoC Foner Kasindi (ZS Mutwanga), of which four validated and two invalidated after investigation; two in Haut-Uélé at the PoC Magambe (ZS Isiro), including one live alert validated and referred to the CTE Tuluba and one lifeless body intercepted, released by security before the swab; three in Bas-Uélé at the PoC PK5 Nesu road (ZS Viadana), all invalidated after investigation.

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Upper Uele	Lower Uele		Global
Report completeness	98,3 % (59/60)	100 % (18/18)	55,5 % (5/9)	85,7 % (6/7)	54,5 % (6/11)	50,0 % (10/20)		83,2 % (104/125)
% of functional strategic PoE/PoC	9,09 % (1/11)	100 %	THAT	35 % (7/20)	54,5 % (6/11)	ND		ND
Number of people who have switched to PoE/PoC	149 443	87 183	7 678	5 510	7 971	3 617		261 402
% of travelers screened	98,0 %	97,91 %	98,3 %	99,3 %	96,0 %	99,4 %		98,0 %
% of travelers who washed their hands	97,4 %	97,91 %	98,3 %	99,1 %	96,0 %	99,4 %		97,6 %
% of travelers made aware	99,3 %	97,91 %	100 %	100 %	96,0 %	99,4 %		98,8 %
Number of alerts notified	3	6	0	0	2	3		14
Number of live alerts validated (suspected cases)	3	4	0	0	1	0		8
Number of bodies recovered today	0	0	0	0	1	0		1
% of suspected cases transferred and CT/CTE	100 % (3/3)	100 % (4/4)	0 %	0 %	100 % (1/1)	THAT		100 % (8/8)
% of lifeless bodies swabbed/secured	0 % (0/0)	0 %	0 %	0 %	0 % (0/1)	THAT		0 % (0/1)
% of suspected cases investigated within 24 h	100 % (3/3)	100 % (4/4)	0 %	0 %	100 % (1/1)	100 % (3/3)		100 % (11/11)

1.2.2. Challenges

Insufficient operationalization of PoE/PoC: national completeness stands at 83.2% (104 out of 125 reports), with low levels in Haut-Uélé (54.5%) and South Kivu (55.5%). In Ituri, only 1 (9.1%) of the 11 strategic PoE/PoCs is operating without interruption, and the Avakubi PoC remains non-functional for security reasons; in Haut-Uélé, five PoCs (Dusu, Apodo,

Aungba, PK6 Route Niangara and Gossamu) did not report their data due to a strike related to the non-payment of service providers; in Tshopo, only 7 out of 20 PoCs are activated.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 52 new positive results** (31 living and 21 deaths) out of 152 new samples received and analyzed (**positivity: 34.2 %**).
- **Tshopo: 2 new positive results out of 12 samples (positivity: 16.7%); official allocation of sampling kits and laboratory consumables by JHPIEGO.**
- **North Kivu : 22 new positive results** (8 living and 14 dead) out of 185 samples analyzed (**positivity of 11.9 %**).
- **Haut-Uélé: 5 new positive results** (5 living and 0 deaths) out of 21 samples analyzed (**positivity of 23.8%**).
- **Bas-Uélé: 2 samples received and tested in Buta, no positive results.**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 31 of the 50 identified rings were opened (62.0%), 65 of the 68 identified households were decontaminated (95.6%), and all 8 health facilities (100%) were decontaminated; 3 health facility assessments were carried out, and 5 infection prevention and control (IPC) kits were distributed to households. The EDS pillar received 87 alerts: 65 bodies were swabbed, 78 EDS were carried out, and 5 were reported at the end of the day.
- **In North Kivu**, 5 out of 10 health facilities (50%) were decontaminated and 48 received support; 7 facilities were assessed (average score 26.7%) and the exposure risk of 4 people with disabilities was evaluated. The EDS (Emergency Drug Safety) pillar received 54 alerts (17 reports): 37 bodies were swabbed, with no failures. For today's alerts, 43 EDS were carried out and 11 were reported.

1.4.2. Challenges

- Weak implementation of PCI standards: in Ituri, ring opening remains limited to 62.0% and swab failures have been recorded; insufficient decontamination teams in Fataki and Mangala and lack of mobility in Komanda, Bambu and Mangala.
- In North Kivu, no decontamination of the places where confirmed cases stayed has been carried out and decontamination teams are lacking in the newly affected health zones.
- In Tshopo, there is a shortage of PPE in health centers and the incinerator at Makiso General Referral Hospital is out of service. Kisangani.

1.5. Continuity of care

1.5.1. Main actions

- **In Ituri**, 111 new admissions were recorded and 85 discharges, including 12 recoveries. Thus, 549 patients are hospitalized out of 978 beds, representing an overall occupancy rate of 56.1%.

- **In North Kivu**, 69 admissions were recorded compared to 33 discharges, including 1 recovery. At the end of the day, 214 patients were hospitalized in 206 beds, representing an overall occupancy rate of 103.9%.
- **In Tshopo**, 2 new admissions of suspected cases and 11 patients isolated for 14 beds, i.e. 78.6% occupancy.
- **In Haut Uélé**, 17 admissions were recorded (3 confirmed and 14 suspected) compared to 15 discharges including 11 recoveries. Thus, 68 patients are in isolation, representing 56.7% of occupied beds.

1.5.2. Challenges

Low reception capacity and absence of CTE in some health zones: saturation of the Bambu CTE (confirmed) and the Kigonze CT (suspected) in Ituri; exceeding the capacity in suspected beds in North Kivu (175 patients for 142 beds), where some of the suspected cases from Butembo, Masereka, Vuhovi and Kalunguta are isolated in ESS which are not treatment centers; in Tshopo, the reception capacity of the CTE is limited to 14 beds.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 270 people were affected around the confirmed cases (67 in Mambasa, 95 in Nia-Nia, 45 in Mangala, 38 in Bunia and 25 in Lolwa); media coverage of the release of 9 recoveries from the Rwankole CTE and 10 recoveries from the Bunia, CME and Elikya CTEs; 46 EDS and swabs facilitated in the ZS of Mangala, Bunia, Tchomia, Komanda, Bambu and Nizi; commitment of the Chief of the Bandjabulu group during a community dialogue in the AS Nzere.
- **In Tshopo**, continued awareness-raising efforts through home visits to 186 households in Makiso. Mangobo and Kabondo.
- **In North Kivu**, briefing of 20 religious leaders in Beni with a guided tour of the CTE and 90 members of the OURAN cultural movement in the ZS of Katwa and Butembo; holding of meetings with national and provincial deputies in Katwa and Butembo; clarification session for education officials; community preparation for decontamination in the ZS of Katwa, Butembo, Kyondo, Beni and Vuhovi.
- **In Haut-Uélé**, awareness and community engagement campaign under the high patronage of the Provincial Governor: 582 people sensitized on August 22 at the official residence and more than 5,200 on August 25 in Ibambi (Wamba territory); unloading of 3 cured people in Ibambi (Pawa Health Zone); 6 cases covered and 5 household heads affected in Isiro and Wamba; 1 community incident resolved in Isiro.
- **In Bas-Uélé**, Production of a radio program with civil society and the CREC task force in Buta and Bondo; reception of awareness-raising materials from FHI 360 (6 speakers, 6 megaphones and 6 USB keys).

1.6.2. Challenges

Persistence of challenges at community level: in Ituri, refusal to sample in Nia-Nia and Alimasi, reluctance in Kabakaba and refusal to list contacts in Kilo State (48 cases); in North Kivu, community resistance in Katwa, Butembo, Kalunguta and Beni and assassination of a president of a community animation cell in the AS Mangothe (ZS of Beni); in Tshopo, resistance to investigations, sampling and transfers to the CTE.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 10 new confirmed cases and 110 confirmed cases under follow-up benefited from SMSPS interventions, as well as 31 new suspected cases and 119 suspected cases under follow-up; 44 households received support, 29 people living with HIV were assisted, 75 children were monitored in the community and 7 in daycare; 47 results were announced, including 10 positive results, and 11 out of 12 recoveries (92%) have been reintegrated. Coverage remains limited to 5 of the 9 health zones that have reported.
- **In North Kivu**, 55.3% of new suspects benefited from SMSPS interventions; 89 results were announced, including 18 positive results, 24 households were supported, 42 PPLs were supported, 30 children were followed up in the community and 4 recovered patients were reintegrated (100%).
- **In Haut-Uélé**, 4 new confirmed cases (3 in Isiro and 1 in Pawa) and 20 confirmed cases under observation benefited from SMSPS interventions, as well as 9 new suspected cases and 1 suspected case under observation; 46 caregivers and 25 people living with HIV received support, 21 results were announced, including 5 positive results, and 1 EDS (Emergency Health Distribution) was supported. • **In Bas-Uélé**, 1 laboratory result was announced in Buta, and 3 people living with HIV received interventions. SMSPS and 1 contact was supported.

1.7.2. Challenges

In Haut-Uélé, SMSPS coverage is insufficient. **In North Kivu**, only 4 health zones are covered by interventions around the latest cases, and there is a lack of childcare facilities in the Ebola Treatment Centers (ETCs). **In Ituri**, there is no coverage in Bambu, Nia-Nia, Mambasa, and Lolwa.

1.8. Logistics

1.8.1. Main actions

Delivery of 40 dignity kits (UNFPA donation) to the Beni health zone and 8 to the Kalunguta health zone, 9 cartons of GF face masks and 20 cartons of PF masks to Beni and 2 cartons of GF masks and 6 PF masks to Kalunguta, as well as supplies for the continuity of care project (UNICEF donation) to the Kalunguta and Butembo health zones. In Tshopo, construction of the Madula and Kabondo isolation units and the large-capacity Ebola Treatment Center (ETC) was completed, equipment was installed for the Kisangani Public Health Laboratory (IMBOLO), and a motorcycle was provided at PK 23 for water supply and thawing vaccines in preparation for vaccination.

1.9 . Security

1.9.1. Main actions

- **In North Kivu**, security coverage of the PCI teams in the AS of Butsili, Madrandele and Kanzunzulili as well as at the Nyakunde CS; accompaniment of the EDS team to the Bundji cemetery for the burial of 4 bodies of confirmed cases and for swabs at the Peregrin CH and the Don Divin CH; security supervision and assessment at the Kyaghala PoE and the Kitakandi PoC.

1.9.2. Challenges

Securing strategic points of presence (PoPs) remains incomplete: in Haut-Uélé, the body intercepted at the Magambe PoP (Isiro Health Zone) was released by security forces without being able to be swabé (a type of body). In North Kivu, insecurity persists in burial areas, and the road leading to the Bundji cemetery (Beni Health Zone) remains impassable.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions.

In Ituri, training and briefing of 22 participants (16 men and 6 women) on the PSEAH during the care training (ZS Bunia); awareness-raising of 128 people in the AS of Mudzimaria (93 participants) and Bigo (35 participants); continuation of the display of messages and the distribution of leaflets in the ZS of Bunia, Aru, Rwampara and Mongbwalu.

1.10.2. Challenges

- Still insufficient coverage of PSEA activities: 25% of the health zones in Ituri (9/36) know the reporting mechanism and only 2 partners (WHO and IMC) report their PSEA activities; lack of mapping of stakeholders; insufficient mass awareness materials; lack of means of transport and computer equipment for the pillar.

MESSAGES CLES

- Ebola transmission remains active, with 81 new cases including 35 community deaths and 7 intra-CTE recorded in the last 24 hours, bringing the national total to 5,794 confirmed cases and 2,786 deaths.
- Two new health zones affected in the province of North Kivu (Biena and Manguredjipa).
- Ituri remains the epicenter of the epidemic, concentrating 82.9% of cumulative confirmed cases and 64.2% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows poor performance (68.0%) of suspected cases investigated while the follow-up rate (82.3%) is below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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